

****HOSPITAL DISCHARGE SUMMARY****

****PATIENT INFORMATION****

Name: Patient 32

Age: 74 years

Sex: Female

MRN: 000-TEST-32

Admission Date: [DATE]

Discharge Date: [DATE]

Length of Stay: 3 days

PRIMARY DIAGNOSIS AND HOSPITAL COURSE

Patient 32 is a 74-year-old female admitted to Metro Medical Center for elective left total hip arthroplasty (THA) secondary to severe primary osteoarthritis of the left hip. Patient had failed conservative management including physical therapy, NSAIDs, and intra-articular corticosteroid injection over the preceding 18 months. Preoperative workup included EKG (showing chronic atrial fibrillation, rate-controlled on current regimen), chest X-ray (unremarkable), and labs within acceptable limits for surgical intervention.

****Operative Course:**** Cemented left total hip arthroplasty performed without intraoperative complications. Prosthesis: cemented acetabular and femoral components. Estimated blood loss 350 mL. Operative time 68 minutes. No perioperative cardiopulmonary events. Post-op imaging (pelvis X-ray) confirms appropriate component positioning.

****Hospital Course:**** Patient extubated in PACU. Pain managed with multimodal analgesia including IV morphine initially, transitioned to PO oxycodone. Ambulated with physical therapy POD#1 (walker, non-weight bearing left lower extremity). Hemodynamically stable throughout; atrial fibrillation remained rate-controlled (HR 60–88 bpm). DVT prophylaxis initiated with enoxaparin 40 mg SC daily. No signs of active bleeding, wound erythema, or warmth. Foley catheter discontinued POD#2; patient voiding spontaneously. Discharged POD#3 to home

with daughter present for assistance.

****PERTINENT MEDICAL HISTORY****

- Atrial fibrillation (chronic, paroxysmal episodes) — rate-controlled
- Obesity (BMI 34.2)
- Hypertension
- Hypothyroidism
- Type 2 diabetes mellitus (diet-controlled)

MEDICATIONS AT DISCHARGE

Medication	Dose	Frequency	Duration
Oxycodone	5 mg	Q 4–6 hours PRN pain	2 weeks
Enoxaparin	40 mg	SC daily	10 days total
Metoprolol	50 mg	BID	Ongoing
Levothyroxine	75 mcg	q.d.	Ongoing
Acetaminophen	650 mg	TID	2 weeks
Docusate	100 mg	BID	2 weeks

DISCHARGE INSTRUCTIONS & ACTIVITY RESTRICTIONS

****Weight-Bearing Status:**** Left lower extremity — ****TTWB (Toe-touch weight-bearing only)**** for 6 weeks, then reassess.

****Hip Precautions (CRITICAL):****

- NO hip flexion >90 degrees
- NO internal rotation of left hip
- NO hip adduction past midline
- Use elevated toilet seat, shower chair
- Sleep with abduction pillow between legs
- Assist with dressing lower body; use reacher

****Ambulation:**** Walker required x 4–6 weeks. Non-slip footwear. Avoid stairs initially; use railing if necessary.

****Wound Care:**** Keep incision clean and dry. Shower allowed after sutures removed (typically POD 10–14). No submersion bathing x 6 weeks. Inspect daily for drainage, warmth, or increased erythema.

****Dietary:**** No specific restriction.